[redacted] False, indisputable

[redacted] False, because I lied/withheld information

[redacted] False/inaccurate, disputable

[redacted] Subjective - projecting their values onto my life

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January 11, 2014

Supreme Court of South Australia
Ground Floor Sir Samuel Way Building
241 – 259 Victoria Square
Adelaide 5000

Attention Presiding Officer Justice P Kelly

Re Giles Patrick CULLITY
Court File Number SCCRM – 13 - 219

Mr. Cullity was seen on January 9th 2014 at Bower Place for the purpose of a report regarding fitness to stand trial.

I advise that I am a legally qualified medical practitioner registered in South Australia as a Specialist Psychiatrist. My medical qualifications are –

- Bachelor of Medicine, Bachelor of Surgery, University of Adelaide 1982
- Fellow of the Royal Australian & New Zealand College of Psychiatry (FRANZCP) 1990
- Member of the Faculty of Forensic Psychiatry RANZCP 2011

I work in private practice, which includes medico legal work and specifically family court work, and as a Senior Consultant Psychiatrist at Flinders Medical Centre. I was an examiner for the Royal Australian and New Zealand College of Psychiatry (RANZCP) for 7 years, and until 2011 was Deputy Chair of the Committee for Specialist International Medical Graduate Education with the RANZCP. I am currently a member of the Appeals Committee of the Royal Australian and New Zealand College of Psychiatry. I continue to have an active role in the assessment and training of psychiatrists across Australia and New Zealand.

I was provided with the documents including

- CBIS summary 9.1.14
- Psychiatric reports Dr Raedside 2.8.13, 23.10.13
- Supreme court case summary
- Witness statements

Profile

Mr. Cullity is a 20 year old man, currently living at home with his parents as he is on home detention. He said he doesn't mind the home detention as he would not be going out anyway. He has no friends, and rarely spends time with family – all his time is spent on developing his ideas and plans. He told me on a number of occasions that Dr. Raeside's conclusions were wrong, wanting to debate these with me, and that he does not have a mental illness.

Current lifestyle, interests and activities

Mr. Cullity told me he spends his time working on a project – wrongly called information technology as it is information systems. He plans this will provide him with a very substantial income within the next 12 months. He told me he is experimenting to see how he can get the most work done in a day. He spends 1 ½ hours asleep, then 2 ½ hours on developing his information systems, then 1 ½ hours asleep, then 2 ½ hours working... He told me chose 1 ½ hours to sleep as this is one complete sleep cycle. When I challenged him that the normal sleep architecture is more complex than this he spoke of how sleep deprived high school students go into REM sleep within minutes, training himself to do this. "My sleep pattern could adapt'. He has occasional breaks from working on his information system project – he explained in detail how he has made a list of activities, ranking them from most to least useful to him in providing a different type of thinking but at the same time still assisting with his main project. 50-75 percent of time is spent on the key project and the rest on other ideas and projects. He takes meal breaks, but not with family as 'their conversation is not interesting and I could be thinking about my own things'. His mother leaves food out and he eats when hungry, with no particular pattern.

When I asked more about the information system he is developing he told me Dr. Raeside didn't know what it is, and that he didn't fully explain it – this would take days. He told me that 'web 3 is the next generation of the internet', and that he is 'trying to rethink the fundamental aspect of personal computing – how information is organised

on personal computers'. He spoke of the tree structure of Windows Explorer, and how large amounts of information make it best to enter key words, but with small amounts the tree structure works. He told me he is designing a way to merge information on the Internet with personal files, planning how to locate information as quickly as possible. He said this is one small aspect.

Mr. Cullity told me he is in the ideation phase, with it not going as fast as he thought. He told me collaboration would be most efficient, but that he needs software to do this, and the current software is not good enough, so he is designing a temporary version to use for collaboration. He said he is designing what it does, right down to how the screen and buttons look, and which ones to click, and what screen each of them then leads to. He said he is designing what it does but wants others to code the program.

He told me that 'if life depended on it I could get this (the information system built and in use) within a year and have (lots of) money. At the rate of a normal person it would be 5 – 10 years'.

Mr. Cullity believes this work with bring a big change to the way people use computers and will make enough money to support him for the rest of his life. He added a maybe, saying he will then work on something else. Later in the interview he told me of a progressive genetic medical condition he believes he has, and that he will make enough money from this information system to develop a cure. He is frustrated that the court issues are taking time away from his work. He does not believe that a conviction will have any impact on his work or future.

In relation to computing experience he told me that whilst he could learn computing at university it would be of no help with this project. I asked about other projects Mr. Cullity alluded to and he said he is working out how to work more productively, with a more efficient Internet, to help organize ideas more efficiently and do other projects. When pressed about these he said he had wanted to design cars and be an auto engineer – but that this is now a hobby, it would mean designing something insignificant, with no time to do what he wants to do. He has been planning and designing what modifications he will make to his car when he owns one – wheel size, 'paint schemes', and a 'drop in engine'. He has also been interested in art, music, design, philosophy and physics. He would like to write music in the future, so when he hears something he likes he will analyse it – the tone of the instruments and note progression. On further questions he used to play drums, and his mother is a primary school music teacher – this

is the extent of his experience.

Information provided indicated that from his teens Mr. Cullity was preoccupied with his eating and weight, and with designing weight training equipment. When specifically questioned he told me he wanted to be muscular and have the physique of a body builder – this commenced in year 9. He read a lot about body building and did weight training, but it didn't have the effect he desired. He wanted to have the body of a body builder to 'get more respect from males and be more attractive to females'. He determined he would work out a better program and devoted himself to doing this. Information provided indicates that by year 11 he would wake 3 times per night to 'feed', measuring food down to the gram, so preoccupied he wasn't attending school. He was working on an exercise program and 'proper weight equipment that no one had designed yet', continually rewriting things on scraps of paper without any apparent order. He could never achieve the goals aimed for with this diet and exercise program and this would make him despondent and he was reported to just sit and not communicate for several days at a time. He planned to build equipment from scrap in the family garage. Mr. Cullity told me that it 'is not uncommon to monitor and modify diet to build muscles, counting every gram and adding up every gram of protein', but he now realizes it was not necessary.

Recently Mr. Cullity has become convinced he has a severe muscle disease. He believes this is Kennedy Disease, but has not been to his GP to have a blood test that could rule this out. He said that if the test is normal it means he has some other serious disease that he hasn't identified yet. When asked about Kennedy Disease he told me it is due to problems with androgen receptors causing low testosterone and degeneration of the nervous system - 'this explains it all'. He said that in this disease the nervous system breaks down over time, with muscle weakness and atrophy, leading to muscle spasm and cramps. He said onset is usually in the 30's – 40's, occasionally earlier, accelerated by strenuous exercise. This he believes is the reason that his body building regime failed. He told me he lost 'sensation' in his muscles since leaving high school – 'they can't contract at the usual level' and he knows this as he 'can't feel it as before'. He believes he is losing strength and size of muscles despite training, so has given up training. He said he knows from looking in the mirror that his muscle size has reduced. Later though he changed this to say that he reached a plateau in body building, and that he had been very slow to build up his muscle bulk. He told me other evidence of him having the condition is that his body shape and young appearance. He believes he now has a feminine body shape – 'my hips are wider, the waist/hip ratio, I have narrow

shoulders, long limbs and a short torso, my pattern of muscle strength, less facial hair and body hair than average'. 'My condition has not progressed to the stage where others would notice'. He believes he has detected it early because he did weights. He went on to tell me about 'apprehension' – that some conditions like Kennedy Disease become more severe in every generation, occurring earlier in each generation, explaining this is why there is no one in earlier generations diagnosed with the condition. He believes he also has it earlier and more severely due to his body building, but that one brother also has it.

Mood symptoms and Anger

Mood disorder has been suggested by others and family report mood problems.
Mr. Cullity reported that from about year 8/9 he would feel down at times, lasting a few hours, but never for lengthy periods of time.

In year 11 Mr. Cullity reported playing well in a Rugby Grand final and feeling 'a bit high' for a day. He said his mood crashed the next day – he wanted to eat, to do with his diet preoccupation, and his father told him to go and do his homework and not worry about food. He said he was annoyed and became suicidal.

He also told me of a music performance in year 10, on drums that went well, and feeling 'slightly high' for the rest of the day. 'Twice in year ~~12~~ 2012 I thought I had an idea that made me a genius. It lasted a day or 2. I read more and realized I was not as smart as I thought'. At these times he said he talked more, had more energy and felt excited, and 'felt grandiose'.

He told me he has had no mood problems for the past 2 years

Mr. Cullity reported that he has a problem with anger, breaking 'stuff' at home. He would not say what. He said his parents do not interfere as much anymore so this is no longer a problem. He believes that 'since they realized I'd live on the streets ... indicating they were interfering quite a bit'. E.g. by asking him to eat meals with the family.

Other history provided suggestive of mental health problems

In 2012 Mr. Cullity became obsessed with a speech that had to be published before 21/12/12 – date of the apocalypse and was fixated on this, not sleeping, saying he would be the next Steve Jobs and a billionaire within 6 months.

Having spoken of his conviction of having Kennedy Disease, or some other serious condition he hasn't yet identified causing the failure of his body building regime, and his absolute rejection of the idea of schizophrenia, schizo affective illness or any other psychiatric condition, Mr. Cullity told me he may have 'another disorder that could explain everything. If it was discovered this was the case I would be forced to change it'. He refused to discuss this any further.

History of events leading to arrest

Mr. Cullity told me that for several months before the stabbing incident he had thought about leaving home and living on the streets. The reason was that he believed he could get more work done on his information system project by doing this. HE said he had no need to live at home, had enough money to live on and would get part time work and work on his project. He did not want to rent anywhere, as this would mean having to spend more time working and having less for the project, hence his plan to live on the street. He told me that his parents 'interfered with my work' by 'imposing things' – such as the expectation he eat meals with them. He wanted them 'off my back'. On the day he left home his mother 'annoyed me' and he 'smashed something' and left. He said he was fine as he had his laptop and wireless Internet, and could charge his laptop at the State Library.

His diet was rigid - a can of tuna, a loaf of bread, fruit and nuts, and milk each day. He had used his savings and planning to look for work. He was not concerned about his personal hygiene, occasionally washing under a sprinkler in the parklands, and occasionally using a Laundromat. He did not think that his level of hygiene would impact on ability to find employment or that he would find it difficult to find work. Initially he had a bicycle, but this 'broke' and he needed to be close to the city.

Mr. Cullity explained how he found a mattress on the other side of the city and carried it to the bridge near the Botanic Gardens where he slept. He had been there 3 nights before the stabbing incident. He reported to me that he returned to his mattress at 3am, having been 'walking around the parklands' with his laptop, working on his project. He said he sensed someone there, having been worried he would find someone. He turned on his bike light and startled the man on the mattress, who appeared annoyed. He said that this man, known now as Daniel Audet, said he initially though he was a policeman, shining the light in his eyes on purpose. He said they walked towards each other and he explained why he had used the light, with Mr. Audet appearing to calm down. They

spoke a little about why each was there, and told me Mr. Audet said he had been kicked out of his accommodation, remarking he seemed drunk and rambled. They then got to the issue of who would spend the night on the mattress. Mr. Cullity said that initially he told him he could have the mattress for just that night. He said he began to walk away, but then thought ' I had it first, why should he get it'. He then began to think that he would lose his spot and have to find another mattress, sleeping on the ground until then. He went back to reclaim his mattress and alleged that Mr. Audet, lying on the mattress, became aggressive and accused him of standing over him and got up, holding a rock, punching it into his other hand and told me to 'go away or I would get my head knocked in'. Mr. Cullity alleged that he backed away and Mr. Audet followed, so he pulled out the knife and said 'you had better back down'. Mr. Cullity said that Mr. Audet was indifferent and not scared, and wouldn't back down. Mr. Cullity said that for him to back down was not an option because if he left he would lose his spot and his mattress permanently, and that if 'I backed down and stayed he would bash me'. He believed that is he fought Mr. Audet without the knife he would be seriously injured. He therefore said he decided to stab him, initially telling me didn't think about where, but then saying he thought 'if I stab him in the torso he might still hit me', therefore stabbing him in the neck. After this he saw Mr. Audet walk towards the hospital, and he walked away, throwing the knife in the river ' I knew I would be in trouble'.

Current Treatment

Mr. Cullity has prescribed olanzapine 10mg and citalopram 10mg since May 2013, but compliance is not monitored. He reported that medication slows him down and impeded his work, but otherwise make no difference

He is seen by a Community Mental Health Team and their notes indicate thoughts of discharge and transfer to a private psychiatrist.

Mr. Cullity complies with psychiatric follow up only because of the court case.

Past History

When asked about suicide attempts or thoughts, or self harm early in the interview Mr. Cullity denied these

Hospital admission May/June 2013 – initial diagnosis of mood disorder revised to paranoid schizophrenia – admission was the result of suicidal ideation and severe agitation following watching a YouTube clip which portrayed 'torture and rape'. He said he thought 'the rest of life will be terrible, and this will outweigh the pleasure', 'knowing it goes on in the world I didn't want to live'. He refused to discuss this

episode any further, saying he had got over it in some way, refusing to elaborate how.

Hospital notes and Dr. Raeside's report are of the suicide attempt in mid 2013, after he watched a YouTube video of graphic images of torture and rape carried out on a movie star he loves 'but she doesn't love him'. The images upset Mr. Cullity who decided the world was so cruel he wanted to kill himself. There is a suggestion from this of possible erotomanic delusions. He had thoughts of killing himself with a chainsaw. On admission his speech was limited, had been wearing ear plugs, had unintelligible writing and been agitated and aggressive at home, but at the same time had grandiose beliefs and plans. It is reported that he said that 'Einstein and Newton lived independently before their great discoveries'. He isolated himself in hospital, rarely leaving his room, and there was no evidence of mania. In this setting FMC diagnosed major depression, and the RAH where he was transferred diagnosed bipolar disorder or schizoaffective disorder, based on the limited reports of mood dysregulation. The grandiosity however was in the absence of mood elevation or a description of past manic episodes, and the periods of low mood very brief and often quite strange.

He was referred to CAMHS in about year 11 but said it didn't help. This was due to feeling 'suicidal' – this appears connected with the issues of body building and lack of success with this.

Family History

Mr. Cullity told me he is the second of 4 boys. He said his youngest brother 16, is still at school and has Asperger's syndrome.

His second youngest brother has just completed year 12 with a perfect score, and is about to commence mechanical engineering at university. He had an episode of 'mania' in the past and was treated with olanzapine for 6 months.

His older brother, 21, is studying engineering.

Mood disorder is reported in his paternal grandfather, a paternal uncle and a maternal uncle. His maternal grandfather was obsessional but had no formal diagnosis.

Forensic History

In year 12 Mr. Cullity sneaked out of home around 1am to meet friends at school to participate in some prank for muck up day. Whilst waiting for them outside the school he told me he saw a police car, so ran off and hid in a garden. A policewoman came down the path with a flashlight and he burst out of his hiding place and tried to make a run for home. He told me the other officer caught him and he thought they were

accusing him of sexual assault, making him very angry, so he resisted arrest. He said it took 4 officers and capsicum spray to contain him, and in the watch house he was really angry thinking they were making things up and out of anger was banging his head on the door, resulting in him being put in a padded cell. He was calm though when his parents arrived and the matter was resolved with a family conference.

Drugs and alcohol

No smoking, alcohol or illicit drug use

Developmental History

Mr. Cullity reported no problems in his early life. Mother is primary school music teacher and father a Professor of Philosophy.

He attended St Ignatius College and hated it, saying they were very controlling with lots of rules. He didn't really like his teachers, had a few close friends and a larger group of other friends, and enjoyed sport. He got into some trouble for fighting. There was no contact with friends outside school.

He gained entry to Marryatville high school, playing drums but not particularly keen on music. He said the atmosphere was much better there, but he had no close friends. His episodes of low mood were mostly in the context of the home setting, but he wouldn't elaborate.

In year 10 he was preoccupied with bulking up his muscles, diet and exercise.

In year 11 he was referred to CAMHS, and then failed year 12 due to his preoccupation with developing programs and equipment to body build.

Mr. Cullity has no friends and has never had a girlfriend. He said he would in the future when in 'better physical condition to have a girlfriend .... Have life where I want it to be .... When rich and successful'. He told me he is going to fix his 'disease thing' (Kennedy Disease) and have a good physique, I will earn lots of money with my software, put it into research and find a cure'.

Understanding of Legal Issues

Mr. Cullity has been to court on a number of occasions and understands the roles of those involved, and the processes in court. He said however that he didn't know if he will plead guilty or not guilty, and asked about what a mental impairment defence would mean. He thought that a suspended sentence would make no difference to his work. He believes that a jail sentence will mean he 'wastes years' of his life in terms of his work, but said he would continue his work paper based.

illness. There have not been significant depressive or manic episodes described, and there is no evidence of either mania or depression at present to account for his presentation on interview and current level of function at home. There has been significant academic and social decline since his mid teens. His manner is quite inconsistent with the content of his beliefs.

A developmental disorder such as Asperger's Syndrome has been suggested – but development early appears unremarkable, with the onset of odd beliefs and behaviour at about 14. If present at all, Asperger's Syndrome is very mild and would not account for the current symptoms and the impact on his life and relationships. Family has also suggested an obsessional personality disorder, but though at a superficial level he is obsessed with his beliefs and these explain all his behaviour, this 'obsession' is because the beliefs are delusional and he is convinced they are correct.

Diagnosis

Paranoid Schizophrenia

Capacity To Commit

I was not asked to report on this, but concur with Dr. Raeside's assessment. It was evident that Mr. Cullity was psychotic at the time of the offence, but he got himself into a position where he had to walk away and give up his spot under the bridge and his mattress, which had taken considerable effort to set up, or go through with his threat after the victim refused to back down when he told him to leave as he (Mr. Cullity) had changed his mind and wanted the mattress back that night after all.
Mr. Cullity had capacity to commit the offence.

Fitness to Stand Trial

Mr. Cullity was able to clearly describe events leading up to and on the night of the alleged offence. He is aware of the roles of those involved in the legal process and in the court room, and also aware of potential outcomes. There was some lack of veracity about events related to his history of mental health symptoms. He continues to have well systematised grandiose delusions and is guarded on some issues. It is difficult to predict how his grandiose delusions will impact on the instructions he provides to the legal team representing him, but based on the interview with me I believe he has capacity to instruct and is therefore fit to stand trial.

Prognosis

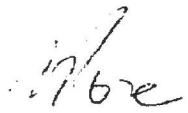
I am concerned that the treating team has failed to recognize the severity of Mr. Cullity's illness and is considering discharge to a private psychiatrist. His delusions are superficially plausible and well systematized, having a profound impact on his life and on his family.

He is prescribed olanzapine but compliance with this is impossible to monitor. Family are avoiding close monitoring or supervision and involvement, as they do not want him leaving home and living on the streets again, or going to jail for breaching conditions. He is left to do as he pleases at home, without interference, rarely talking even to family, by his own admission.

I would be surprised if Mr. Cullity is taking the olanzapine regularly, as he would be unable to do the pattern of sleep 1 ½ hours, work 2 ½ hours, sleep 1 ½ hours etc. – because of the sedating effect of this medication. By his own admission it impacts on his productivity, and he would do anything to avoid having this compromised – even live on the streets.

Unless treatment is changed, and compliance monitored, there is likely to be no change in his condition over the next 12 months.

Yours sincerely



Lynette Rose